

CH SHARIF AND SAEED HOSPITAL

Department Billing Split & Settlement v7.2

Separate department invoices, central patient collection, payment allocation and outsourced-provider settlement.

FINAL SOURCE OF TRUTH - VERSION 7.2

Admission begins at Front Desk. Department bills remain separate. Central collections are allocated department-wise. Doctor discharge re-authentication, outsourced department settlement, Panel receivables, high-cost medicine approval, doctor-sponsored discount and doctor tax rules are included.

Supersedes v7.1 for all revised admission, billing, department settlement, doctor finance and panel-flow rules.

1. Department Split Principle

One patient may have many department invoices. Front Desk can show one consolidated statement, but accounting ownership remains department-specific.

Department	Example gross	Owner-specific discount/contract	Net bill
Hospital Services	PKR 20,000	Hospital-funded discount only if approved	Department net
Laboratory	PKR 20,000	Lab/provider or panel contract rule only	Department net
Pharmacy	PKR 10,000	Pharmacy/contract rule only	Department net

2. PKR 50,000 Worked Split

- Hospital Services: PKR 20,000
- Laboratory: PKR 20,000
- Pharmacy: PKR 10,000
- Total patient bill before discounts/coverage: PKR 50,000.

If patient pays the full PKR 50,000 at Front Desk, the receipt is allocated PKR 20,000 to Hospital Services, PKR 20,000 to Laboratory and PKR 10,000 to Pharmacy. Physical collection is central; invoice ownership is not merged.

3. Hospital Discount Example

Hospital Services = PKR 20,000. Hospital-funded discount = PKR 5,000. Hospital Services net becomes PKR 15,000. Lab and Pharmacy remain unchanged unless they have their own approved contract/owner-specific adjustment. New total becomes PKR 45,000.

4. Partial Payment Allocation

Invoice	Outstanding before	Allocation from PKR 20,000 payment	Outstanding after
Hospital Services	25,000	10,000	15,000
Laboratory	12,000	5,000	7,000
Pharmacy	18,000	5,000	13,000

5. Advance Handling

- Advance received at admission is patient credit.
- It may remain unallocated while no department charge exists.
- As charges are posted, Front Desk may allocate available advance to selected department invoices.
- Advance itself is not immediately counted as Lab/Pharmacy/Hospital revenue before allocation.

6. Outsourced Provider Payable

Ledger item	Rule
Billed	Department invoice amount exists; does not by itself mean provider is payable in cash.
Patient collected	Patient share actually collected and allocated to that department.
Panel realized	Panel remittance actually received and allocated to that department.
Realized base	Patient collected + panel realized, subject to provider contract basis.
Provider payable	Amount eligible for settlement after contract adjustments/withholding if configured.
Settled	Full/partial amount already paid to provider.

Ledger item	Rule
Outstanding	Provider payable - settled amount.

7. Settlement Controls

- Billing/Admin/Super Admin can settle according to permissions.
- Payment methods: Cash, Bank Transfer, Cheque, Online/Other configured method.
- Settlement voucher stores department/provider, period, eligible realized amount, settlement amount, method/reference, representative name/designation, settled by, received/acknowledged by, date/time and remarks.
- Partial settlement preserves remaining payable.